NCCP 00212 · VERSION 7

Bevacizumab every 14 days · breast treatment component

Bevacizumab infusion on Day 1 of each 14-day cycle with separately prescribed paclitaxel. Continue while the cancer is controlled and treatment is tolerated, with regular review; there is no fixed number of breast-treatment cycles.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Raised blood pressure and protein in the urine

☐ Bleeding, blood clots and problems with wound healing

☐ Rare bowel perforation, abnormal internal connections called fistulae, or serious wound infection

☐ Rare severe headache, confusion, seizures or vision changes associated with a brain syndrome

☐ Infusion reactions; fatigue and other effects may be increased by the chemotherapy partner

Discuss disease-control goals and alternatives, the lack of established overall-survival improvement in the supporting paclitaxel study, blood-pressure and proteinuria monitoring, bleeding and clotting, bowel perforation/ fistula, wound healing, infusion reactions, reproductive precautions and the additional risks of paclitaxel.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: blood count, kidney and liver tests, urine protein, blood pressure and a heart history/examination. Repeat blood count, kidney/liver tests and urine testing before each cycle; check blood pressure before treatment and after it. Extra checks depend on symptoms and results.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending